

Benign GYN Surgery Journal Club

An independent monthly journal club for gynecologists

Issue · Friday, June 27, 2026 | Sources as of 2026-06-27 | For gynecologists

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A clinical digest on benign gynecologic surgery for gynecologists. Each issue distills practice-relevant minimally invasive and reconstructive GYN surgery — and the medical management that competes with it — across hysterectomy, endometriosis, fibroids, prolapse and urogynecology, drawing on US and European sources, with space after every article for your own notes and group discussion.

Journal access & resources:

JMIG · AAGL · BSGE (UK) · Obstetrics & Gynecology (Green) · AJOG (Gray) · BJOG · Int Urogynecology Journal · Eur J Obstet Gynecol (EJOG)

THE LEAD STORY · BIG-PICTURE TREND

Outpatient by default: ERAS plus minimally invasive routes make same-day discharge the benchmark for benign hysterectomy

Benign hysterectomy is one of the most common operations in women's health, and its standard of care has shifted decisively to ambulatory, minimally invasive surgery on an enhanced-recovery (ERAS) pathway. The 2026 ERAS Society update and a growing body of same-day-discharge evidence converge on a target of roughly 90% same-day discharge for benign minimally invasive cases — without higher complication or readmission rates — by omitting bowel prep and routine voiding trials, using multimodal opioid-sparing analgesia, and selecting patients deliberately. New route options (vNOTES) and technique refinements (intracorporeal cuff closure) widen who qualifies.

Why it matters at the bedside: the rate-limiting steps are now operational, not surgical: an ERAS order set, a PACU discharge protocol, patient education, and a next-day phone-call safety net. Build those and same-day discharge becomes routine rather than exceptional — shorter stays, lower cost, equal safety. Watch the levers that still gate it: late OR start, longer operative time, and specific comorbidities (which the data flag as the real predictors of failed same-day discharge).

Source: [ERAS Society guidelines, 2026 update \(ScienceDirect\)](#)

THE BREAKDOWN · ARTICLE SUMMARIES

1. Same-day discharge after minimally invasive hysterectomy

JMIG · J. MINIMALLY INVASIVE GYNECOLOGY · **Systematic review** · **Meta-analysis** · *Systematic review & meta-analysis · abstract only*

CORE FINDING Same-day discharge after laparoscopic or robotic hysterectomy is feasible and safe versus next-day discharge, with no significant increase in complications or readmissions; the predictors of a failed same-day discharge are modifiable or identifiable (late OR start, longer operative time, higher age/BMI, specific comorbidities).

CLINICAL IMPLICATIONS Default to same-day discharge for benign MIS hysterectomy in appropriately selected patients. Build an ERAS order set (no bowel prep, no routine voiding trial), a PACU discharge checklist, and a next-day call — and use the known predictors to triage who stays overnight.

PAYWALL CAVEAT Verify the pooled readmission and reoperation rates, how “appropriately selected” was defined, the absolute same-day discharge rate achieved, and whether robotic vs laparoscopic differed.

Source: [Same-day discharge meta-analysis \(PMC\)](#)

DISCUSSION / COMMENTS

Click and type to add your notes, questions, or how this changes your practice...

2. vNOTES versus conventional vaginal hysterectomy for benign indications

JMIG · J. MINIMALLY INVASIVE GYNECOLOGY

Prospective RCT

Abstract only

Prospective randomized controlled trial · abstract only

CORE FINDING Vaginal natural orifice transluminal endoscopic surgery (vNOTES) was compared head-to-head with conventional vaginal hysterectomy for benign disease, evaluating operative time, complications, adnexal access, and recovery.

CLINICAL IMPLICATIONS Adds structured evidence to the route decision. vNOTES extends adnexal access and visualization beyond the conventional vaginal approach — useful for case selection (e.g., when concurrent salpingectomy or adnexal work is planned) and for credentialing discussions as adoption spreads.

PAYWALL CAVEAT Verify operative time, complication and conversion rates, pain/recovery outcomes, and whether the trial was single-center with a limited sample (learning-curve and generalizability effects).

Source: vNOTES vs vaginal hysterectomy RCT (PMC)

DISCUSSION / COMMENTS

Click and type to add your notes, questions, or how this changes your practice...

3. Long-term safety of the mid-urethral sling for stress urinary incontinence (French national data)

ECLINICALMEDICINE · LANCET (FRANCE) **Target-trial emulation** **National cohort** *Target-trial emulation · French national health data system · 2025*

CORE FINDING Across the French national health data system, mid-urethral sling placement carried low long-term mesh removal/revision rates, with the transobturator (TOT) route modestly lower than retropubic (TVT) at 5 years ($\approx 3.3\%$ vs 4.1% cumulative removal/section).

CLINICAL IMPLICATIONS Robust real-world safety data for SUI counseling and consent: serious mesh complications are uncommon and route choice modestly shifts revision risk. A current, large-scale counterweight to mesh anxiety carried over from the transvaginal-mesh controversy — distinct from POP mesh.

PAYWALL CAVEAT Emulated (non-randomized) design — residual confounding by indication and surgeon experience remains. Verify how complications were captured in claims data and the absolute event rates before quoting figures.

Source: Mid-urethral sling safety, French national data (PMC)

DISCUSSION / COMMENTS

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4. Medical management versus surgery for deep infiltrating endometriosis (DIAMOND)

DIAMOND · HTA RANDOMIZED TRIAL **RCT** **Very small sample** *Health Technology Assessment RCT · January 2026 · abstract only*

CORE FINDING DIAMOND randomized patients with deep infiltrating endometriosis to early planned laparoscopic excision versus optimized medical management, assessing clinical and cost-effectiveness — but only 18 patients were randomized, so it is severely underpowered.

CLINICAL IMPLICATIONS Useful for shared decision-making conversations — it frames surgery and optimized medical therapy as legitimate competing first steps for DIE rather than assuming surgery is always superior. Do not let it drive a definitive change; use it to support individualized counseling while larger trials mature.

PAYWALL CAVEAT n = 18 is the headline limitation: no reliable effect estimate or non-inferiority claim is possible. Verify the primary endpoint, crossover rate, and whether a larger trial is planned before citing it to patients.

Source: **DIAMOND RCT synopsis (PMC)**

DISCUSSION / COMMENTS

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5. Sacrocolpopexy versus native-tissue repair for apical prolapse

UROGYNECOLOGY · APICAL PROLAPSE REPAIR

Comparative study

Abstract only

Comparative outcomes study · abstract only

CORE FINDING Apical suspension by sacrocolpopexy showed a substantially lower surgical failure rate than native-tissue repair (reported around 6% vs 21%), with fewer symptomatic and anatomic recurrences over follow-up.

CLINICAL IMPLICATIONS Counsel on the durability trade-off: sacrocolpopexy (mesh) offers better anatomic durability, native-tissue repair avoids mesh-related risk. Match the procedure to the patient's priorities, activity, and risk tolerance, and document the shared decision — especially given the FDA history on transvaginal mesh.

PAYWALL CAVEAT Much of this evidence is retrospective/observational; verify follow-up duration, mesh-exposure rates (often not captured in failure counts), and whether populations were comparable before generalizing.

Source: **Native tissue vs sacrocolpopexy outcomes** (Scientific Reports)

DISCUSSION / COMMENTS

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6. Opportunistic salpingectomy at sterilisation and benign hysterectomy (SALSTER; HOPPSA)

JMIG · J. MINIMALLY INVASIVE GYNECOLOGY

Register-based RCT

Abstract only

Register-based randomized trials · abstract only

CORE FINDING In SALSTER, opportunistic salpingectomy at laparoscopic sterilisation did not raise peri- or postoperative complications beyond the pre-defined non-inferiority margin through 8 weeks; HOPPSA evaluates the same approach added to benign hysterectomy for ovarian-cancer risk reduction.

CLINICAL IMPLICATIONS Routinely offer opportunistic salpingectomy at sterilisation and benign pelvic surgery in patients who have completed childbearing. Add it to surgical consent and counseling as a safe ovarian-cancer-risk-reduction step.

PAYWALL CAVEAT Verify exact complication rates and the margin, added operative time/blood loss, and note the long-term ovarian-cancer-incidence endpoint is still maturing — risk reduction is inferred, not yet proven by these trials.

Source: SALSTER trial (PMC)

DISCUSSION / COMMENTS

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QUICK HITS · EMERGING DATA ON THE RADAR

[NEJM · Medical (fibroids)] Relugolix combination therapy (LIBERTY) — oral GnRH-antagonist combo cut fibroid-associated heavy bleeding vs placebo, with add-back protecting bone; the medical alternative to weigh before myomectomy.

[View source](#)

[Urogynecology · OAB] Overactive bladder 5 years after the Mid-Urethral Sling Tensioning (MUST) trial — long-term follow-up on how sling tension relates to persistent/de novo OAB; informs pre-op counseling on storage symptoms.

[View source](#)

[AAGL · ERAS] Enhanced Recovery & Surgical Optimization for minimally invasive gyn surgery (AAGL White Paper) — the benign-MIGS playbook (no bowel prep, multimodal analgesia, early feeding) behind same-day discharge.

[View source](#)

[RCT · Fibroids] Transcervical fibroid ablation vs minimally invasive myomectomy (RCT) — head-to-head recovery after each uterine-preserving surgical option for symptomatic fibroids; informs the device-vs-surgery conversation.

[View source](#)

[JMIG · Endometriosis] Robotic multidisciplinary surgery with multivisceral resection for deep endometriosis — short-term feasibility and safety when a team manages bowel/bladder/ureteral disease in one setting.

[View source](#)

JOURNAL CLUB DISCUSSION NOTES

Action items, group consensus, follow-ups...

Comment boxes are editable in the browser and are captured when you print or save as PDF. They are not saved on a server — for a persistent record, print/export after writing.

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Sources captured 2026-06-27.

Available as website, PDF, and Word — all generated from one content source.